

Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (90 FR 60108)

Last updated February 2026



February 23, 2026

Electronic Submission via Regulations.gov

Department of Health and Human Services

Office of the Deputy Secretary

Assistant Secretary for Technology Policy

Office of the National Coordinator for Health Information Technology

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (90 FR 60108)

Dear Sir or Madam:

Homecare Homebase (HCHB) appreciates the opportunity to comment on the Department's Request for Information regarding actions HHS can take to accelerate the adoption and use of artificial intelligence (AI) in clinical care.

HCHB provides an enterprise electronic health record (EHR) and care coordination platform serving home-based care providers, including home health and hospice agencies, that collectively care for approximately one million patients daily and represent approximately 47.1 percent of Medicare home health and 43.5 percent of Medicare hospice volume. Our customers operate at the intersection of hospital discharge, chronic disease management, serious illness care, and end-of-life support in the home. The populations they serve are clinically complex, and their risk drivers frequently extend beyond diagnosis codes to include functional limitations, caregiver availability, home environment constraints, longitudinal hospitalization patterns, and medication complexity. AI adoption in clinical care will depend less on algorithmic capability and more on regulatory clarity, operational predictability, interoperable data access, and governance frameworks that reflect the realities of care delivery.

Executive Summary

HCHB supports responsible AI adoption that strengthens clinical oversight, reduces administrative burden, and improves care coordination, particularly for medically complex patients receiving services in the home. Accelerating AI deployment will require regulatory clarity, proportionate governance frameworks, interoperable access to clinically meaningful data, and predictable program integrity expectations.

This letter outlines recommendations in five areas:

- (1) providing regulatory clarity so organizations can invest in AI with confidence and maintain workforce stability;
- (2) explicitly supporting AI tools that assist clinicians with documentation while preserving clinician review and accountability;
- (3) applying stronger oversight to higher-risk AI uses and lighter guardrails to lower-risk, workflow-support tools;
- (4) using AI to identify billing anomalies while preserving human review and appeal rights in coverage and payment decisions; and
- (5) improving data exchange and research alignment so AI can better identify risk and support earlier, preventive intervention.

HCHB also emphasizes the importance of ensuring that home-based care providers are fully represented in AI policy development, interoperability initiatives, and governance design. With clear guardrails and balanced oversight, AI can enhance quality, access, and cost containment across the care continuum.

Regulatory Predictability, Capital Allocation, and Workforce Stability

(Response to Questions 1 and 7)

Technology procurement in healthcare is inherently risk-adjusted. Boards and compliance leaders evaluate AI tools not only for potential clinical or operational benefits, but also for exposure to audits, recoupment risk, malpractice liability, and False Claims Act implications. When federal expectations regarding AI governance are unclear, organizations defer investment.

Regulatory ambiguity increases procurement friction, lengthens sales cycles, and raises the effective cost of capital for both providers and developers. It also slows innovation. Uncertainty discourages investment in low-risk, workflow-supportive AI that could meaningfully reduce administrative burden and improve care coordination. In the absence of clear federal guardrails, organizations may avoid deploying tools that are clinically helpful but perceived as legally uncertain.

For home-based care providers, AI-enabled systems increasingly function as core infrastructure for managing clinical risk, financial accountability, and preventable utilization. These providers operate within tight cash-flow cycles, paying clinicians weekly while reimbursement may lag. Unpredictable or opaque AI-driven expectations, whether related to documentation review, audit analytics, or reimbursement interpretation, can destabilize staffing and limit access in rural and underserved markets. Predictability, therefore, is not simply an operational preference; it is a workforce and access imperative.

In addition, payment methodologies for home-based care incorporate statutory productivity adjustments intended to reflect system-wide efficiency gains. While AI may support workflow efficiencies, productivity assumptions derived from acute-care environments may not translate directly to home-based settings, where travel time, interdisciplinary coordination, caregiver engagement, and functional assessment drive workload in ways not reducible through automation alone. Policymaking that assumes uniform productivity gains across care settings risks overstating achievable efficiencies in decentralized, patient-home environments. Governance and payment frameworks should reflect the operational realities of home-based care to ensure stability and continued access.

AI-Assisted Documentation and OASIS Clarification

(Response to Question 2)

Documentation burden in home-based care directly affects clinician retention, visit capacity, and patient access. AI-assisted documentation presents a meaningful opportunity to reduce burden while strengthening internal consistency and audit defensibility.

In the home health context, CMS guidance provides that “[a]n agency’s software may not ‘answer’ or ‘generate’ the OASIS response for the assessing clinician.” This requirement appropriately reinforces that OASIS responses must reflect the clinician’s assessment of the patient’s current status. At the same time, clarification is warranted as AI-enabled documentation capabilities evolve.

There is a meaningful distinction between software that independently determines and enters an OASIS response and systems that generate draft language or structured suggestions based on contemporaneous clinical inputs for clinician review and attestation. In appropriately governed AI-assisted workflows, the clinician remains responsible for evaluating suggested content, making any necessary modifications, and formally confirming the final response. Audit trails can document that process clearly.

From a practical standpoint, the public interest is not served by requiring that clinicians manually type or dictate every element of documentation. Patients and regulators appropriately expect that clinicians review, validate, and attest to the accuracy of clinical notes and care plans. Governance language that implies manual data entry as the benchmark for compliance risks impeding safe AI adoption. The critical safeguard is clinician accountability and verification, not the mechanical method of input.

Clarifying that AI-assisted documentation support, subject to active clinician review, is distinguishable from automated completion would preserve assessment integrity while enabling responsible innovation.

Program Integrity, Administrative Sophistication, and Contestability

(Response to Question 6)

AI can play a constructive role in identifying billing anomalies and unusual utilization patterns. AI-enabled anomaly detection, when applied to objective billing and utilization patterns with appropriate human review, can strengthen program integrity and reduce improper payments. However, there is an important distinction between anomaly detection based on claims patterns and retrospective second-guessing of clinical documentation through automated review. As AI capabilities expand, their use in program integrity efforts warrants careful calibration. HCHB does not oppose the use of advanced analytics to identify bad actors. However, documentation authorship or drafting method alone should not be treated as a risk signal.

High-quality clinical documentation, whether manually authored or AI-assisted, can be well structured, internally consistent, and detailed. Conversely, fraudulent actors can mimic the appearance of thorough documentation. It is therefore unlikely that effective fraud detection will hinge on having AI systems “read charts” for stylistic similarity or drafting patterns. Documentation style, linguistic consistency, or drafting efficiency should be neutral inputs, not proxies for compliance risk.

Home-based care providers frequently serve medically and socially complex patients whose longitudinal needs naturally result in detailed and structured documentation over time. AI systems that rely on linguistic similarity, structural consistency, or documentation density as indicators of aberrancy risk generating false positives and disproportionately burdening compliant providers serving high-acuity or end-of-life populations.

Program integrity analytics are most effective when focused on substantive discrepancies between documented clinical presentation, service intensity, and billing patterns, rather than on the technological tools used to prepare documentation.

Where AI is used to triage audits or flag aberrant patterns, meaningful human oversight and clear pathways to contest or appeal AI-generated findings must be preserved. Contestability safeguards are essential to maintaining trust, protecting beneficiaries, and preventing erroneous adverse actions against compliant providers.

Risk-Differentiated Governance, Explainability, and Guardrails

(Response to Questions 3, 4, and 5)

AI architectures may leverage foundation models, domain-specific models, or hybrid approaches; however, federal policy should focus on governance, explainability, and workflow

integration rather than the specific underlying model design. Not all AI systems present equivalent clinical or compliance risk. Workflow-support tools operating under clinician oversight differ materially from autonomous clinical decision systems or AI implementations that influence payment, compliance, or quality measurement.

Governance expectations should be proportionate to use-case risk and address validation, bias assessment, transparency, monitoring, and model drift management.

For lower-risk tools such as clinical summarization or documentation support, practical guardrails can meaningfully reduce risk without imposing excessive burden. These may include requirements that AI-generated summaries preserve consistency with source data, suppress unsupported inferences, and avoid introducing or “inventing” protected health information not grounded in the clinical record. Such guardrails protect factual integrity while preserving flexibility for innovation.

HHS may consider working collaboratively with industry stakeholders to develop guardrail frameworks for common low-risk use cases. Shared guardrail principles would promote consistency and trust while allowing technology to evolve responsibly.

Where AI influences documentation expectations, compliance review, quality reporting, or payment-related workflows, explainability is essential. Providers should have access to high-level explanations of the factors influencing outputs, documentation of how variables were selected, and sufficient transparency to understand how clinical documentation aligns with system behavior. In this context, explainability supports balanced provider participation and consistent documentation standards, not coding optimization.

For higher-impact AI implementations affecting payment, compliance, or standardized reporting, phased implementation or multi-year parallel testing prior to operational consequences would allow evaluation of divergence, unintended impacts, and operational disruption before full integration.

Governance frameworks should remain proportionate so as not to disadvantage smaller or rural providers without corresponding safety gains.

Interoperability and Data Fragmentation

(Response to Question 8)

If AI is expected to contribute meaningfully to cost containment, it must have timely access to the costliest and most clinically consequential datapoints in the system: hospital admissions, discharges, emergency department visits, and other material changes in patient status. Home-based care providers cannot prevent avoidable utilization if they do not know, in near real time, that a hospitalization has occurred or that a clinically significant transition is underway. Without

interoperable Admission/Discharge/Transfer (ADT) visibility, AI-enabled prevention becomes reactive rather than anticipatory.

In many markets, critical transition data remains confined within referral portals or proprietary care coordination platforms. When key patient indicators require manual portal access rather than operating as interoperable, event-level signals across the continuum of care, AI cannot function as preventive infrastructure. Broader operationalization of Trusted Exchange Framework and Common Agreement (TEFCA) and other standardized exchange frameworks should prioritize real-time ADT exchange and shared visibility into clinically material events across care settings.

Yet even if exchange infrastructure improves, data fragmentation across care settings remains a structural barrier to true visibility. Most AI models are trained primarily on acute-care datasets and do not generalize well to post-acute or home-based populations. In these settings, risk is driven less by diagnosis codes and more by functional status, caregiver availability, medication complexity, home environment constraints, and longitudinal instability. When these factors are underrepresented in both training data and exchange frameworks, predictive accuracy declines, particularly for the populations most dependent on proactive intervention.

Data fragmentation is compounded by two forces: essential patient information is often inaccessible due to interoperability barriers, and the data that is available frequently lacks the structured, comprehensive elements required to train reliable post-acute care AI models.

Patient Protection and High-Impact AI Use

(Response to Question 9)

The policy implications of AI in clinical care depend on whether its use affects workflow or alters care decisions.

AI applications that support documentation accuracy, risk identification, or care coordination operate within clinician workflow and generally do not independently change the trajectory of care nor the patient's perception of care quality. By contrast, AI systems that influence service authorization, discharge timing, visit frequency, prioritization, or resource allocation have the potential to result in care disruption.

Similarly, when designed with real-time data retrieval, clinician oversight, and transparent rationale generation, AI-enabled prior authorization tools can reduce delays and administrative burden while preserving individualized medical necessity determinations.

In home-based settings where patients are medically fragile and reliant on coordinated in-home services, even modest disruptions in service intensity or timing can precipitate avoidable hospitalization or institutionalization. Governance frameworks should therefore distinguish clearly between workflow-support AI and AI that shapes high-impact care determinations.

Where AI influences decisions that affect access to covered care, accountability must remain clearly assigned to licensed professionals or accountable entities, and affected determinations should be subject to meaningful review. AI policy should reinforce the principle that technology strengthens coordination and clinical stability rather than introducing new sources of fragmentation or unintended service disruption.

Research Priorities, Preventive Supports, and MLR Alignment

(Response to Question 10)

HHS can further accelerate responsible AI adoption by supporting applied research that connects AI-enabled identification of preventive and wellness needs to coverage policy and Medical Loss Ratio (MLR) treatment.

Home-based care providers routinely identify upstream drivers of avoidable utilization, including nutritional risk and caregiver instability. Enteral nutrition delivered via PEG tube is generally covered, while medically fragile individuals who require oral supplemental nutrition to prevent deterioration may lack comparable coverage pathways. Similarly, caregiver strain is a documented predictor of hospitalization and institutionalization, yet structured caregiver support may not clearly qualify as medical expense under current MLR frameworks.

AI-enabled tools can generate actuarially relevant evidence linking preventive supports to measurable reductions in acute care spending. Supporting research in this area would allow policymakers to evaluate whether clinically indicated preventive supports, when algorithmically validated and outcome-linked, warrant clearer recognition within medical expense definitions and coverage structures. Aligning preventive intervention coverage with MLR treatment would reduce structural disincentives and encourage investment in upstream care that supports aging in place and long-term cost containment.

Ensuring Home-Based Care Is Not Left Behind

Home health and hospice providers have historically been underrepresented in federal health information technology policy. Neither setting was eligible for Meaningful Use incentive payments under HITECH, and neither has benefited from a tailored ONC certification framework comparable to those applicable to hospitals and eligible professionals. As a result, home-based providers have often invested in health IT infrastructure without the same level of federal financial support, standardized certification pathways, or interoperability alignment afforded to facility-based settings.

These structural differences matter. Home-based care is central to national goals related to cost containment, aging in place, serious illness management, and hospital capacity management.

Yet policy frameworks are frequently designed with acute-care delivery assumptions that do not fully reflect decentralized, in-home care environments.

As AI governance evolves, it is important that home-based care not be further disadvantaged through standards, interoperability requirements and gaps, validation methodologies, or oversight mechanisms that assume facility-based workflows or data structures. Inclusion of home-based care stakeholders in governance design, pilot programs, interoperability initiatives, and research efforts will be essential to ensuring modernization across the continuum of care.

AI policy presents an opportunity not only to accelerate innovation, but to correct historical imbalances in federal health IT support. Thoughtful inclusion of home-based care in AI strategy will strengthen system-wide performance and improve outcomes for medically complex populations served outside traditional institutional settings.

Conclusion

Home-based care AI has the potential to serve as core infrastructure for managing risk, improving preventive intervention, and supporting aging in place. Realizing that potential will require regulatory clarity, interoperable data access, proportionate governance expectations, explicit explainability standards, thoughtful phased implementation for high-impact uses, and alignment between payment policy and clinically validated upstream interventions.

With predictable and transparent governance, AI can function not as a compliance risk, but as a force multiplier for quality and cost containment.

Thank you for the opportunity to comment. We welcome the opportunity to provide additional data or operational details to support HHS's ongoing work. Should you have any questions, feel free to contact me at 214-239-6778 or lukerutledge@hchb.com.

Respectfully submitted,

A handwritten signature in black ink, appearing to read 'Luke Rutledge', with a long horizontal flourish extending to the right.

Luke Rutledge, President

Homecare Homebase